

COMPREHENSIVE DISCHARGE SUMMARY

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Facility	Example Medical Centre — Doha, Qatar
Document type	Discharge Summary — Internal Medicine (5 pages)
Patient	Mr. Ahmed Al-Karim (fictional)
Date of birth	14 February 1944
MRN	SAMPLE-2048-DOH
Admission	10 December 2026
Discharge	17 December 2026
Consultant	Dr. Khalid Rahman (fictional)

1. Presenting complaint

Seven-day admission for community-acquired pneumonia with delirium. Treated with intravenous then oral antibiotics; delirium resolved by day five. Mobility temporarily reduced; physiotherapy input during admission. (Fictional.)

2. Problem list

- Community-acquired pneumonia — resolving
- Delirium — resolved
- Dementia, Alzheimer type
- Chronic heart failure (HFpEF)
- Type 2 diabetes
- Hypertension
- Constipation
- Low mood (sertraline started November 2026)

3. Hospital course (detail)

Day 1–2. Oxygen 2 L via nasal cannula, IV ceftriaxone, IV fluids. Confused at night, sitter provided.

Day 3–4. Fever settled. Oxygen weaned. Swallow assessment normal. Started mobilising with frame.

Day 5–6. Oral amoxicillin-clavulanate. Delirium resolved; MMSE back to 19/30 baseline. Dietitian: mild weight loss, supplement drinks started.

Day 7. Home with family. Discharge weight 70.9 kg.

4. Investigations

Date	Test	Result
10 Dec	Chest X-ray	Right lower zone consolidation
10 Dec	CRP	148 mg/L
16 Dec	CRP	12 mg/L
16 Dec	Potassium	4.1 mmol/L (normal)

5. Medications on discharge

Medication	Dose	Directions	Status	Until
Amoxicillin-clavulanate	625 mg	1 tablet three times daily with food	NEW	21 Dec 2026 (complete course)
Nutritional supplement drink	1 bottle	twice daily between meals	NEW	review at follow-up
Sertraline	50 mg	1 tablet each morning	continue	
Memantine	10 mg	1 tablet once daily	continue	
Donepezil	5 mg	1 tablet at night	continue	
Bisoprolol	2.5 mg	1 tablet each morning	continue	
Furosemide	40 mg	1 tablet each morning	continue	
Metformin	500 mg	1 tablet twice daily with meals	continue	
Lactulose	10 ml	twice daily if constipated	continue	
Melatonin	2 mg	at bedtime	STOPPED	stopped 12 Dec (delirium)

Paracetamol 500 mg may be used up to four times daily if needed.

6. Instructions for family and carers

- Complete the antibiotic course; do not stop early even if he feels well.
- Encourage fluids and the supplement drinks; weigh weekly and record.
- Continue daily weights for heart failure (call HF nurse if +2 kg in 3 days).
- Sit upright for meals; cough or breathlessness returning → same-day GP/clinic review.
- Delirium can recur with infection or dehydration: new confusion → seek medical review.
- Physiotherapy home exercises sheet attached (page 5).

7. Follow-up appointments

What	When	Where
Internal medicine post-discharge review	7 January 2027, 13:30	Building A, clinic 3
Repeat chest X-ray	on or after 4 January 2027	Radiology, ground floor (walk-in 08:00–14:00)
Geriatric Medicine (existing)	19 January 2027, 08:45	Building A, room 7
Neurology (existing)	14 January 2027, 11:15	Neurology clinic room 4
Heart failure nurse phone review	week of 28 December 2026	nurse will call family

Contacts (fictional): Ward 4 East +974 4000 0000 ext. 4400; HF nurse ext. 1234.

8. *Physiotherapy home exercise sheet*

- Sit-to-stand from a firm chair: 5 repetitions, twice a day.
- Marching on the spot holding the counter: 1 minute, twice a day.
- Walk indoors with frame: 5–10 minutes, three times a day, supervised.
- Deep breathing: 5 slow breaths every hour while awake for the first week.

Dr. Khalid Rahman (fictional) — Consultant, Internal Medicine — 17 December 2026